Example 2 — Sepsis From Complicated Urinary Tract Infection With Obstructing Stone

Patient: 64-year-old woman
Admission diagnosis: Sepsis secondary to complicated UTI/pyelonephritis
Discharge diagnoses:

- Sepsis due to complicated urinary tract infection
- Left ureteral calculus with hydronephrosis
- Status post ureteral stent placement
- Acute kidney injury, resolved
- Type 2 diabetes mellitus
- Hypertension

Hospital Course

The patient presented with fever, rigors, left flank pain, nausea, and dysuria. Initial laboratory testing demonstrated leukocytosis, elevated lactate, and acute kidney injury. CT abdomen/pelvis demonstrated a left ureteral stone with associated hydronephrosis.

She received IV fluids and empiric broad-spectrum antibiotics. Given concern for infected obstruction, urology performed urgent left ureteral stent placement for decompression. Urine culture subsequently grew a susceptible strain of *E. coli*. Blood cultures remained negative. Antibiotics were narrowed based on culture and susceptibility results.

Renal function returned near baseline following decompression and treatment of infection. The patient remained afebrile and hemodynamically stable for >24 hours before discharge.

The ureteral stent remains in place and requires outpatient urologic management for definitive stone treatment and eventual stent removal.

Discharge Medications

New medications

- **Cefpodoxime 200 mg by mouth twice daily for 7 additional days** to complete antibiotic therapy.
- **Tamsulosin 0.4 mg by mouth once daily** while ureteral stent remains in place, unless otherwise directed by urology.
- **Acetaminophen as needed** for mild pain or fever, within the maximum daily dose instructed by the treating team.

Continue

- Amlodipine 5 mg daily.
- Atorvastatin 20 mg nightly.
- Home diabetes medications as reviewed on the final medication reconciliation.

Stopped/held

- Lisinopril — temporarily held pending outpatient renal function and blood-pressure reassessment.
- NSAIDs — avoid because of recent kidney injury unless specifically approved.

Discharge Instructions

- Complete all prescribed antibiotics.
- Maintain adequate oral hydration unless another clinician has specifically restricted fluids.
- Follow up with urology for definitive stone management and ureteral-stent removal/exchange.
- Do not assume that absence of pain means the stent can remain indefinitely; it requires planned follow-up.
- Have kidney function rechecked as directed.
- Monitor blood glucose according to the established diabetes plan.

Return Precautions

Return immediately for:

- Fever or shaking chills.
- Worsening flank or abdominal pain.
- Persistent vomiting or inability to keep medications down.
- Confusion, fainting, severe weakness, or difficulty breathing.
- Markedly decreased urine output.
- Symptoms suggesting recurrent urinary obstruction or infection.

Nursing Discharge Teaching / What Nursing Typically Relays

Nursing would typically emphasize:

- **Finish the antibiotic exactly as prescribed.**
- Explain common urinary symptoms associated with a ureteral stent, while distinguishing these from warning signs of infection.

- Confirm that the patient knows the **urology follow-up date or scheduling process**.
- Reinforce hydration instructions.
- Review medication changes and specifically identify medications that were stopped or held.
- Teach the patient to monitor for fever and worsening urinary/flank symptoms.
- Review glucose monitoring if relevant to the patient's diabetes plan.
- Confirm that prescriptions were obtained or that the patient knows where/how they will be filled.
- Use teach-back to confirm understanding of antibiotic duration and return precautions.